MARSHALL, Jeffrey (id #30081, dob: 10/21/1957)

Note: A Summary of Care Record (SCR) for this patient will be sent separately.

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[ID:1152048-H-4331]

Referral Order

03/24/2026

To Provider	From Provider
AVERY HOME HEALTH **5660 W FLAMINGO RD STE A** **LAS VEGAS, NV 89103** **Phone:** **Phone: (702) 416-0611** **Fax:** **Fax: (702) 899-4479**	**MAIN OFAI** **3175 SAINT ROSE PKWY STE. 320** **HENDERSON, NV 89052-3500** **Phone: (702) 997-9833** **Fax: (702) 666-0413**

Referral Order Information

Diagnosis	• **Postoperative wound dehiscence, subsequent encounter** **ICD-10: T81.31XD: Disruption of external operation (surgical) wound, not elsewhere classified, subsequent encounter**
Order Name	**Orders included: 1** **Postoperative wound dehiscence, subsequent encounter** **ICD-10: T81.31XD: Disruption of external operation (surgical) wound, not elsewhere classified, subsequent encounter** • **WOUND CARE REFERRAL** **Schedule Within: provider's discretion Note to Provider: please initiate wound vac therapy on a non-healing diabetic foot wound and perform MWF**
Notes	**please initiate wound vac therapy on a non-healing diabetic foot wound and perform MWF**

Patient Information

Patient Name	**MARSHALL, JEFFREY**
Sex - DOB - Age	**M 10/21/1957 68yo**
Address	**5025 KELL LN** **LAS VEGAS, NV 89115-6703**
Phone	**H: (307) 277-6300** **M: (307) 277-6300**
Primary Insurance	***SELF PAY***
Secondary Insurance	**None recorded.**

Electronically Signed by: AUSTIN SOLOMON, DPM

MARSHALL, Jeffrey (id #30081, dob: 10/21/1957)

Encounters and Procedures

Clinical Encounter Summaries

Encounter Date: 03/24/2026

Patient

Name	MARSHALL, JEFFREY (68yo, M) ID# 30081	**Appt. Date/Time**	03/24/2026 10:15AM
DOB	10/21/1957	**Service Dept.**	MAIN OFAI
Provider	AUSTIN SOLOMON, DPM		
Insurance	Med Primary: *SELF PAY* Prescription: SURESCRIPTS LLC - This member could not be found in the payer's files. Please verify coverage and all member demographic information. details		

Chief Complaint

left foot

Vitals

None recorded.

Allergies

Reviewed Allergies

Medications

None recorded.

Problems

Reviewed Problems
- Postoperative wound dehiscence, initial encounter - Onset: 03/02/2026
- Cellulitis of toe of left foot - Onset: 03/02/2026
- Type 2 diabetes mellitus with diabetic peripheral angiopathy with gangrene, unspecified whether long term insulin use - Onset: 03/02/2026

Family History

Family History not reviewed (last reviewed 03/02/2026)

Social History

Social History not reviewed (last reviewed 03/02/2026)

Surgical & Procedure History

Surgical & Procedure History not reviewed (last reviewed 03/02/2026)

Past Medical History

Past Medical History not reviewed (last reviewed 03/02/2026)

Screening

Name	Score	Notes
STEADI Fall Risk	Not scored	

HPI

Patient is a 68yoM who presents today following left partial 5th ray amputation due to gangrene on 2/13/26. He

MARSHALL, Jeffrey (id #30081, dob: 10/21/1957)

ambulates in normal shoegear today - no surgical shoe as his last one broke. No new complaints. No recent N/V/F/C/SOB/CP.

ROS

None recorded.

Physical Exam

Examination of the left lower extremity reveals lightly palpable DP/PT pulses. Gross sensation is not intact. Gross motor function is intact. Surgical site is clean and dry. No erythema noted around the distal lateral aspect of the incision. Incisions are approximated with dehiscence noted at the distal aspect which is now a full-thickness wound with a fibrogranular base. No direct probe-to-bone. No active drainage. No areas of fluctuance or crepitus.

Procedure Documentation

BS Wound debride:

Sharp, excisional debridement was performed today in the office. Debridement was carried down to, and including, the level of subcutaneous tissue. All necrotic, fibrotic, and non-viable tissue was removed until the wound base exhibited healthy, bleeding, granular tissue. Hemostasis was achieved with pressure to the wound and a dressing was then applied consisting of wet to dry dressing.

Assessment / Plan

68yoM, diabetic, presenting for a postoperative visit following left partial 5th ray amputation on 2/13/26.

The surgical site was evaluated and is healing with full-thickness dehiscence on the distal aspect of the incision, without clinical sign of infection. The patient was instructed to continue WBAT in CAM boot/post op shoe. Given the depth of the wound, I will prescribe **negative pressure wound therapy (wound vac)**, **a wound care clinic performing vac changes MWF**. Wound care instructions were reinforced, including keeping the area clean and dry and monitoring for any signs of infection such as redness, drainage, or increasing pain. A gradual transition plan was outlined, with anticipated local wound care until complete healing. He requested to go back to work - he states he now has a sitting scooter he can use so he does not have to walk around as much. This is okay as long as he wears his CAM boot when he does have to ambulate. If he has any worsening symptoms, we will likely have to keep him off work for a longer duration. The patient was advised to **follow up in 2 week** for repeat evaluation. All questions were addressed, and the patient verbalized understanding of the recovery timeline and activity restrictions.

1. **Cellulitis of toe of left foot**
   L03.032: Cellulitis of left toe

2. **Postoperative visit**
   Z48.89: Encounter for other specified surgical aftercare

3. **Type 2 diabetes mellitus with diabetic peripheral angiopathy with gangrene, unspecified whether long term insulin use**
   E11.52: Type 2 diabetes mellitus with diabetic peripheral angiopathy with gangrene

4. **Postoperative wound dehiscence, subsequent encounter**
   T81.31XD: Disruption of external operation (surgical) wound, not elsewhere classified, subsequent encounter

- WOUND CARE REFERRAL - Schedule Within: provider's discretion Note to Provider: please initiate wound vac therapy on a non-healing diabetic foot wound and perform MWF

Return to Office

- AUSTIN SOLOMON, DPM for FOLLOW UP at MAIN OFAI on 04/07/2026 at 10:30 AM

Encounter Sign-Off

Encounter signed-off by AUSTIN SOLOMON, DPM, 03/24/2026.

Encounter performed and documented by AUSTIN SOLOMON, DPM
Encounter reviewed & signed by AUSTIN SOLOMON, DPM on 03/24/2026 at 01:41 PM